

1. LUNG CANCER - Chest x-ray suggestive of lung cancer (including pleural effusion and slowly resolving consolidation). - Persistent haemoptysis in smokers/ex smokers over 40 years of age. - High suspicion of lung cancer with normal x-ray. - History of asbestos exposure, recent onset chest pain, shortness of breath, or where chest x-ray indicates pleural effusion, pleural mass or any suspicious lung pathology. For other lung symptoms consider urgent chest x-ray first. REFER IMMEDIATELY PATIENTS WITH - Signs of superior vena cava obstruction (swelling of the face/neck with fixed elevation of jugular venous pressure). - Stridor.	CODE 1A 1B 1C 1D	3. LOWER G.I. CANCER All ages - A definite palpable right-sided lower abdominal mass. - A definite palpable rectal (not pelvic) mass. Aged 40 and over - Rectal bleeding WITH a change in bowel habit to looser stools and/or increased frequency persistent for ≥ 6 weeks. Over 60 years - Rectal bleeding persistently WITHOUT anal symptoms or change in bowel habit. - Change of bowel habit to looser stools and/or increased frequency WITHOUT rectal bleeding and persistent for ≥ 6 weeks. Any Age - Iron deficiency anaemia WITHOUT an obvious cause (Hb $< 11\text{g}/100\text{ml}$ in men or $< 10\text{g}/100\text{ml}$ in postmenopausal women) Please ensure that a full blood count, abdominal, or rectal examination has been carried out as appropriate prior to referral.	CODE 3A 3B 3C 3D 3E 3F	5. GYNAECOLOGY CANCER - Clinical features suggestive of cervical cancer on examination. - Vulval bleeding due to ulceration or an unexplained vulval lump. - Palpable pelvic or abdominal mass not obviously fibroids or of gastrointestinal or urological origin. - Postmenopausal bleeding where patient not on hormone therapy, or where patient taking tamoxifen. - Persistent intermenstrual bleeding and negative pelvic examination. - On HRT with unexpected or prolonged bleeding persisting for more than 6 weeks after stopping HRT.	CODE 5A 5B 5C/D 5E 5F 5G
2. UPPER G.I. CANCER - Dysphagia – food sticking on swallowing (any age). - Dyspepsia at any age combined with one or more of the following 'alarm' symptoms - weight loss - proven anaemia - vomiting - epigastric mass - dysphagia - chronic gastrointestinal bleeding - suspicious barium meal result - Unexplained persistent recent onset dyspepsia in a patient aged 55 years or more. - Obstructive jaundice - Upper abdominal mass without dyspepsia. - Unexplained upper abdominal pain and weight loss, with or without back pain. CONSIDER urgent referral for patients with: - Unexplained worsening of dyspepsia combined with: - Barrett's oesophagus - Peptic ulcer surgery over 20 years ago - Known dysplasia, atrophic gastritis, intestinal metaplasia. - Persistent vomiting and weight loss in the absence of dyspepsia. - Unexplained weight loss or iron deficiency anaemia in the absence of dyspepsia.	2A 2B 2C 2E 2F 2I 2D 2G 2H	4. BREAST CANCER - Female patients with a lump: - any age – with a discrete, hard lump with <u>fixation</u>, with or without skin tethering. - aged ≥ 30 years – with a discrete lump that persists after their next period or presents after menopause. - aged < 30 years – with a lump that enlarges, is fixed and hard, or in whom other reasons for concern apply e.g. family history. - Previous breast cancer patients – with a further lump or suspicious symptoms. - Patients with one of the following nipple problems: - unilateral eczematous skin or nipple change that does not respond to topical treatment. - nipple distortion of recent onset. - spontaneous unilateral bloody nipple discharge. - Male patients, aged ≥ 50 years with a unilateral, firm subareolar mass with or without nipple distortion or associated skin changes.	4A 4B 4C 4D	6. UROLOGICAL CANCER - Macroscopic Haematuria in adults. - Microscopic Haematuria in adults over 50 years. - Swelling or mass in the body of the testis. - Abdominal mass identified clinically or on imaging, thought to arise from urinary tract. - Recurrent or persistent urinary tract infection associated with haematuria in adults aged ≥ 40 years. - An elevated age specific PSA in men who are not significantly compromised by other co-morbidities. - A high PSA ($> 20\text{ng/ml}$) in men with a clinically malignant prostate. (PSA result should accompany referral) - Any suspected penile cancer. - With symptoms and high PSA levels.	6A 6B 6C 6D 6E 6F 6G 6H 6I